



TEST REPORT

Name : **Mr. PUROHITH MANOJ SINGH**
Age/Gender : **26 Years / Male**
Registration ID : **260120137872**
Ref. By : **Dr. SRIDHAR G.**
Sample Type : **Serum**

Registered on : 22-Mar-2026 13:06
Collected on : 22-Mar-2026 13:42
Released on : 22-Mar-2026 16:27
Printed on : 22-Mar-2026 17:54
Regn Centre : **Dilsukhnagar - 12**

CREATININE

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Creatinine	: 4.6	mg/dL	0.7 - 1.2
Method: Jaffe Kinetic IDMS traceable			
e-GFR (Glomerular Filtration Rate)	: 16.7	ml/min/1.73 m ²	>= 90
Method: Calculation, CKD EPI equation			

Interpretation / Comments :

- Serum Creatinine is useful in the diagnosis of renal insufficiency and is more specific and sensitive indicator of renal disease than serum Urea /BUN.
- Use of simultaneous Urea / BUN and creatinine levels provide more information in the diagnosis of renal insufficiency.
- Amount of Creatinine produced in the body is relatively constant (unlike Urea) and is primarily a function of muscle mass (Creatinine being a waste product formed in muscles.).
- Age, gender, Race, Muscle mass, Exercise, Certain Drugs, diet, Dehydration are amongst the physiological factors affecting serum creatinine concentration.
- GFR (Glomerular Filtration Rate) is generally considered the best index of overall kidney function. Repeated determination in conjunction with creatinine assay establish whether the patient has stable or progressive disease.
- The CKD-EPI equation is the most widely used IDMS traceable equations for estimating GFR in patients above 18 years of age. This equation includes variables for age and gender, and it may be observed that Kidney may be involved despite a serum creatinine concentration appearing to be within or just above the Biological Reference Interval. The results of e-GFR by CKD-EPI equation are normalized to 1.73 m² body surface area. CKD-EPI equation is not valid for individuals under 18 years of age.
- Estimates for GFR based on serum creatinine will be less accurate for patients at the extremes of muscle mass (such as frail elderly, critically ill, cancer patients) and also those with unusual diets, sudden acute renal failure, patients on dialysis and patients with severe liver disease.
- Confirmatory tests with exogenous measured GFR or directly measured creatinine clearance should be performed for such individuals.
- The influence of creatinine measurement imprecision at low creatinine concentrations (and hence high e-GFR) has a possible contribution to the variability at higher e-GFR values.



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VIJAYA DIAGNOSTIC CENTRE®

3-6-16 & 17, Street No. 19, Opp. Lane to Tanishq, Chandra Nagar, Himayatnagar, Hyderabad - 500029

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Registration ID : **260120137872**
Ref. By : **Dr. SRIDHAR G.**
Sample Type : **Serum**

Registered on : 23-Mar-2026 12:04
Collected on : 23-Mar-2026 12:52
Released on : 23-Mar-2026 16:40
Printed on : 23-Mar-2026 17:23
Regn Centre : **Dilsukhnagar - 12**

BICARBONATE

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
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Bicarbonate	: 18.1	mmol/L	21 - 31
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Method: PEPC

Interpretation / Comments :

- Bicarbonate is a significant indicator of electrolyte dispersion and anion deficit
- Together with pH determination, it is used in the diagnosis of and treatment of various disorders associated with acid base balance.



MC-2657

DR. N. SUJANA
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Registration No: 50478



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Opp.Metro Pillar No. A1516, Indira Nagar, Dilsukhnagar, Hyderabad - 500089

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ELECTROLYTES

<u>TEST NAME</u>	<u>RESULT</u>	<u>UNIT</u>	<u>BIOLOGICAL REFERENCE INTERVAL</u>
Sodium <i>Method: Indirect ISE</i>	: 142	mmol/L	136 - 146
Potassium <i>Method: Indirect ISE</i>	: 4.5	mmol/L	3.5 - 5.1
Chlorides <i>Method: Indirect ISE</i>	: 111	mmol/L	101 - 109

Interpretation / Comments :

- Sodium : Levels of sodium when evaluated with electrolytes aid in assessing acid base balance, water balance and water intoxication.
- Potassium : Useful in evaluation of electrolyte balance, cardiac arrhythmia, muscular weakness, hepatic encephalopathy and renal failure.
- Chloride : Useful when assayed along with sodium, potassium and bicarbonate in assessment of electrolyte, acid base and water balance.



DR. T KINNERA
MD BIOCHEMISTRY
Registration No: 69554



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Name : **Mr. PUROHITH MANOJ SINGH**
Age/Gender : **26 Years / Male**
Registration ID : **260120137872**
Ref. By : **Dr. SRIDHAR G.**
Sample Type : **Whole Blood - EDTA**

Registered on : **22-Mar-2026 13:06**
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COMPLETE BLOOD PICTURE (CBP)

<u>TEST NAME</u>	<u>RESULT</u>	<u>UNIT</u>	<u>BIOLOGICAL REFERENCE INTERVAL</u>
Haemoglobin <i>Method: Photometric Measurement</i>	: 8.9	gm/dL	13.0 - 17.0
Total RBC Count <i>Method: Coulter Principle</i>	: 2.9	millions/cumm	4.5 - 5.5
Packed Cell Volume / Hematocrit <i>Method: Calculated</i>	: 26.9	%	40.0 - 50.0
MCV <i>Method: Derived From RBC Histogram</i>	: 92.6	fL	83.0 - 101.0
MCH <i>Method: Calculated</i>	: 30.6	pg	27.0 - 32.0
MCHC <i>Method: Calculated</i>	: 33.1	gm/dL	31.5 - 34.5
RDW <i>Method: Derived From RBC Histogram</i>	: 16.8	%	11.6 - 14.0
Total Leucocytes (WBC) Count <i>Method: Coulter Principle</i>	: 4900	Cells/cumm	4000 - 10000
<u>Differential count</u>			
Neutrophils <i>Method: VCS 360 Technology and Microscopy</i>	: 65	%	40 - 80
Lymphocytes <i>Method: VCS 360 Technology and Microscopy</i>	: 25	%	20 - 40
Eosinophils <i>Method: VCS 360 Technology and Microscopy</i>	: 2	%	1 - 6
Monocytes <i>Method: VCS 360 Technology and Microscopy</i>	: 8	%	2 - 10
Basophils <i>Method: VCSn Technology and Microscopy</i>	: 0		0-2
<u>Absolute Leucocyte Count</u>			
Absolute Neutrophil Count <i>Method: Calculated</i>	: 3185	Cells/cumm	2000 - 7000
Absolute Lymphocyte Count <i>Method: Calculated</i>	: 1225	Cells/cumm	1000 - 3000
Absolute Eosinophil Count <i>Method: Calculated</i>	: 98	Cells/cumm	20 - 500



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COMPLETE BLOOD PICTURE (CBP)

<u>TEST NAME</u>	<u>RESULT</u>	<u>UNIT</u>	<u>BIOLOGICAL REFERENCE INTERVAL</u>
Absolute Monocyte Count <i>Method: Calculated</i>	: 392	Cells/cumm	200 - 1000
Platelet Count <i>Method: Coulter Principle</i>	: 162000	Cells/cumm	150000 - 410000
<u>Peripheral Smear</u>			
RBC <i>Method: Microscopy of Leishman stained smear</i>	: Normocytic Normochromic with anisocytosis		
WBC <i>Method: Microscopy of Leishman stained smear</i>	: Normal in morphology, maturity and distribution		
Platelets <i>Method: Microscopy of Leishman stained smear</i>	: Adequate		



MC-6432

**DR. VARSHINI
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MD PATHOLOGY**
Registration No: TSMC/FMR/08778

----- End of Report -----

* Suggested Clinical Correlation, If Necessary Kindly Discuss with signatory